Spatial Analysis

May 4, 2023

1 Methods

For each sample, we will compare the distribution of each markers in the normal and lesion regions.

- Markers for DD: CD3, CD4, CD8a, CD11c, CD15, CD45, CD68, CD69, CD123, CD279
- Markers for CF: CD1a, CD3, CD4, CD8a, CD11c, CD15, CD45, CD69, CD123, CD279 (CD68 did not pass QC)

For CD123, segmentation is done at flow=0.5 to get more masks in the bright area and a threshold of 5000 is set for total intensity to remove masks in the debris field in the lesion area.

The size of the skin tissue lesion area is determined by morphology using wheat germ agglutinin (WGA) flourescent stain. ImageJ is used to demarcate the lesion area, then a straight line ROI is drawn to span from one side of the lesion to the other side of the lesion, with the endpoints of the line positioned at the outermost skin boundary.

Some considerations on region boundary and distance metric:
1. Should we draw the boundary between normal and lesion based on WGA only or all markers?
2. Shall each ROI be convex?

For (1), the two choices lead are answering different questions. Drawing boundary based on WGA only answers a stronger question than drawing the boundary based on all markers.

For (2), a convex shape is a shape that a line connecting any two points in the shape completely lies within the shape. Practically speaking, in this context it is equivalent to drawing a straight line to separate normal from lesion. Pros for straight boundary: the span between any cell and the closest point on the skin lies completely within the region. So the distance measure is well defined. In contrast, if a line from a cell to its closest point on the skin surface cuts through both normal and lesion regions, the interpretation of distance to skin surface is muddled. Pros for curved boundary: Regions (which is a type of neighborhoods) do not necessarily have geometric shapes.

These two questions are also intertwined. If we use WGA only to draw boundaries, we can only use straight line boundaries because the difference between normal and lesion is only visible in epidermis in WGA staining. A straight line can be drawn in epidermis and extend to dermis, but not a curve.

2 Results

	Counts				Counts/mm2			
	normal		lesion		normal		lesion	
	epi	dermis	epi	dermis	epi	dermis	epi	dermis
CD3	74	1222	158	5395	631	2874	121	1399
CD15	11	238	6567	2867	93	559	5050	743
CD123	131	879	1350	8038	1117	2067	1038	2084
CD11c	343	2056	8029	17905	2926	4836	6174	4643
CD68	154	998	2524	11081	1314	2347	1941	2874
area	0.12	0.43	1.30	3.86				

Table 1: DD

	Counts				Counts/mm2			
	normal		lesion		normal		lesion	
	epi	dermis	epi	dermis	epi	dermis	epi	dermis
CD3	422	8435	331	5629	976	4476	819	5574
CD15	7	2154	756	391	16	1143	1871	387
CD123	76	3680	324	1298	175	1952	801	1285
CD11c	624	8498	1308	4299	1443	4509	3237	4257
area	0.43	1.88	0.40	1.01				

Table 2: CF

	DD	CF	DD/CF
lesion width	2862	1202	2.38
interval between dist to skin peaks	607	427	1.42
interval between dist to DEJ peaks	319	337	0.95
CD3 epi	121	819	0.15
CD3 dermis	1399	5574	0.25
CD3 e/d	0.09	0.15	0.60
CD15 epi	5050	1871	2.70
CD15 dermis	743	387	1.92
CD15 e/d	6.80	4.83	1.41
CD123 epi	1038	801	1.30
CD123 dermis	2084	1285	1.62
CD123 e/d	0.50	0.62	0.81
CD11c epi	6174	3237	1.91
CD11c dermis	4643	4257	1.09
CD11c e/d	1.33	0.76	1.75

Table 3: Comparing the space between CD3 and CD15 peaks with lesion size. Distance is measured in um, size is measured in square mm.

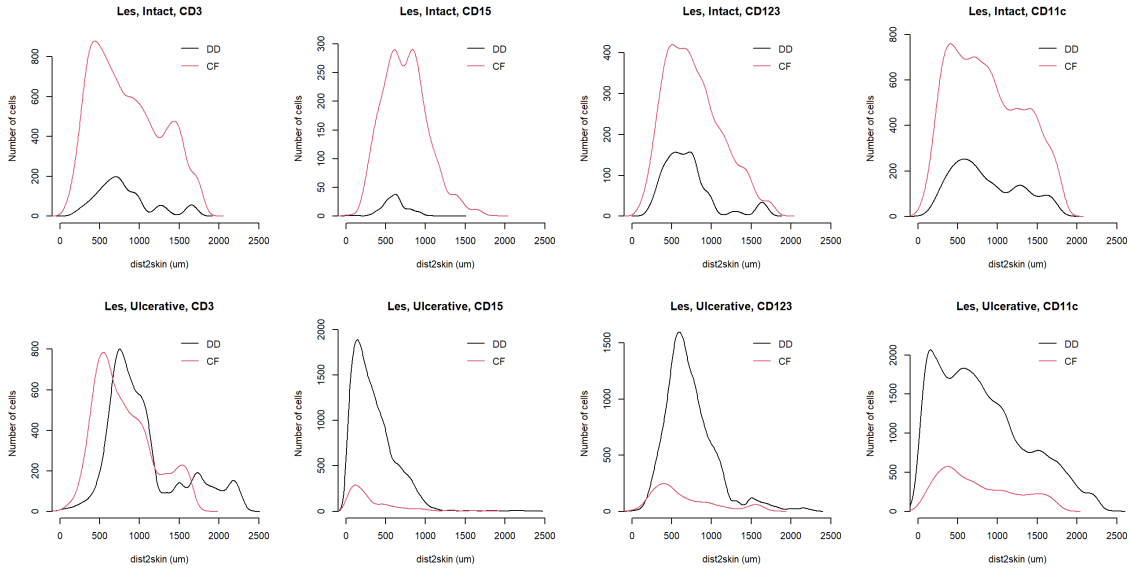


Figure 1: Distance to skin, counts.

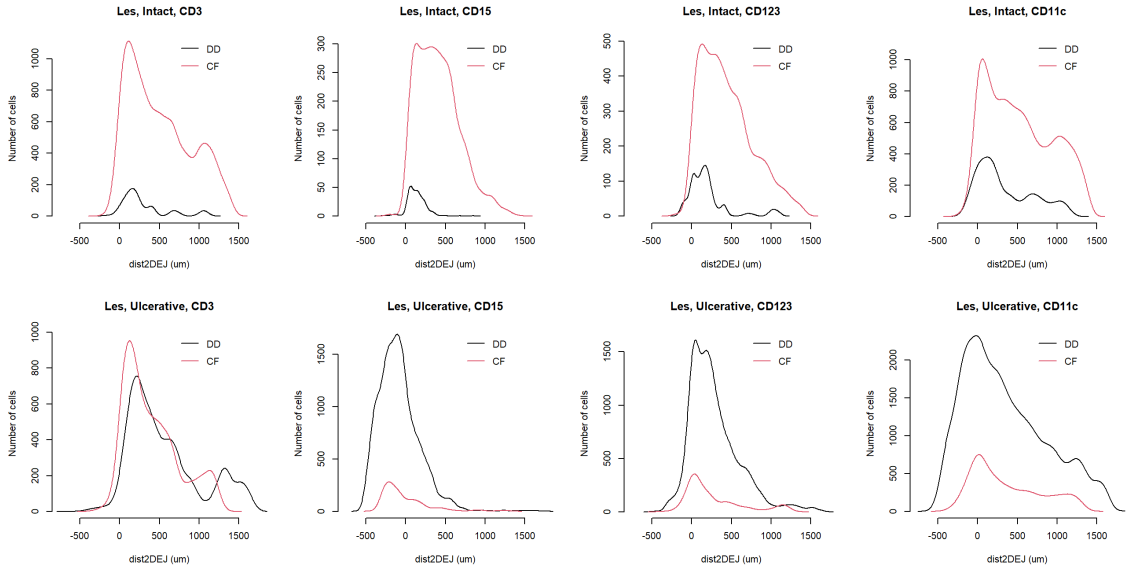


Figure 2: Distance to DEJ, counts.

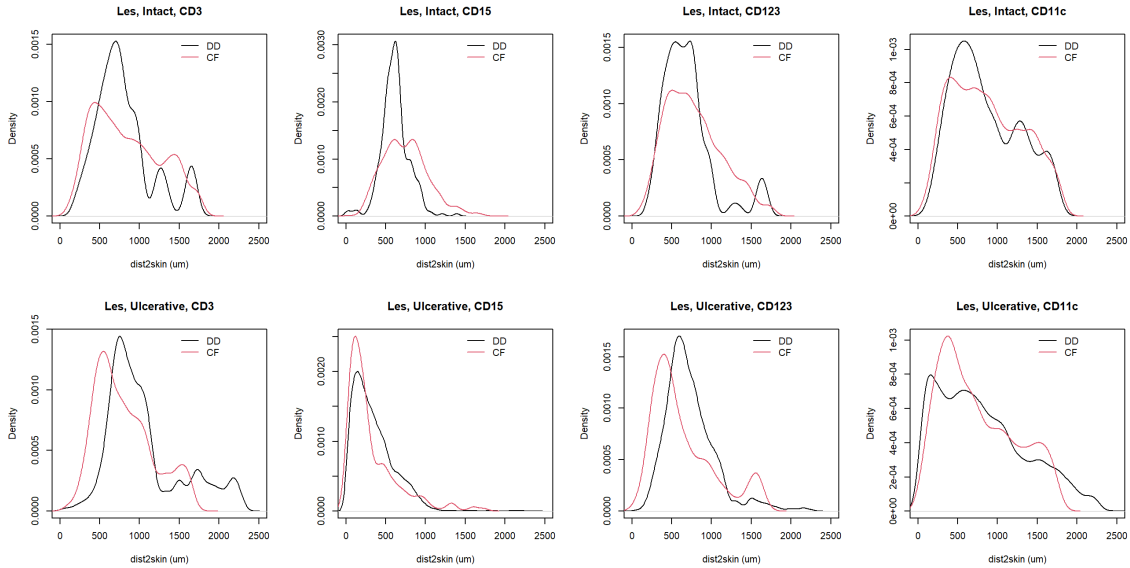


Figure 3: Distance to skin, density.

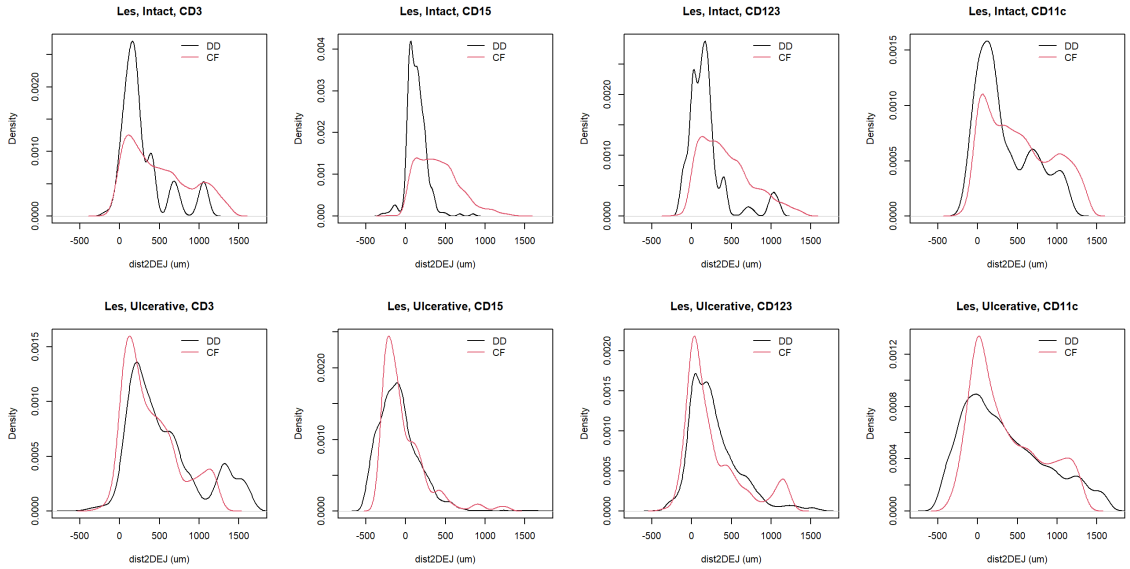


Figure 4: Distance to DEJ, density.

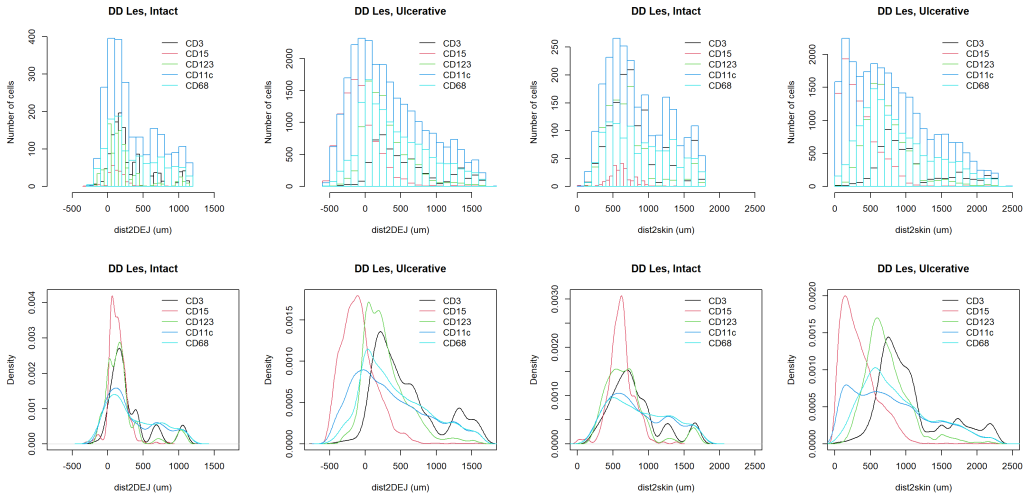


Figure 5: Distance to DEJ (left) and skin surface (right).

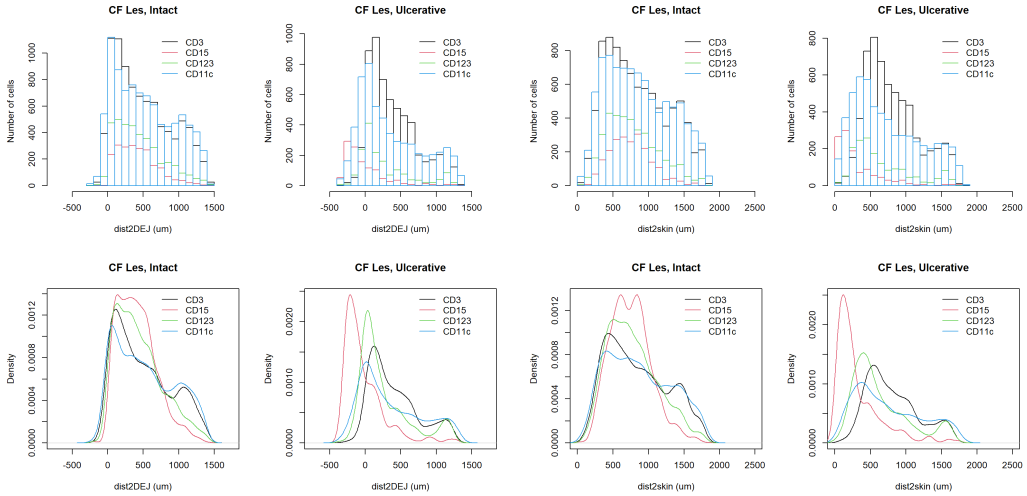


Figure 6: Distance to DEJ (left) and skin surface (right).

3 Appendix